

[IEEE GLOBECOM 2026] Your paper #1571254564 ('6G Network empowering AI agents')

2 messages

IEEE Globecom 2026 SAC - EH <ieeeglobecom2026sac-eh-chairs@edas.info>

Fri, Jul 31, 2026 at 7:44 PM

To: Ayush Debnath <ayush.d@kgpian.iitkgp.ac.in>, Ruelia Saha <ruelia@kth.se>, Sudip Misra <smisra.editor@gmail.com>

Dear Mr. Ayush Debnath:

Congratulations! We are pleased to inform you that your paper #1571289364 entitled "OmniMed-FL: A Robust Multimodal Federated Learning Framework for Clinical Diagnosis" has been ACCEPTED for presentation at 2026 IEEE Global Communications Conference: Selected Areas in Communications: E-Health. This is a significant achievement, considering less than 40% of submissions were accepted at IEEE GLOBECOM 2026.

***Review and Feedback**

The review reports for your paper are at the end of this email. Alternatively, they can be accessed via EDAS at <https://edas.info/showPaper.php?m=1571289364>.

***Registration**

To ensure publication in the IEEE GLOBECOM 2026 Proceedings and submission to IEEE Xplore®, at least one author must register for the conference at either the Full or Limited author registration rate. This non-refundable fee is required before uploading the final formatted version of your paper. Author registration will open on the IEEE GLOBECOM 2026 website soon, and an email will be sent when it is open. For authors with multiple accepted papers, one Full or Limited registration is valid for up to three (3) papers by the same author.

***Paper Revision and Final Manuscript Upload**

Each accepted paper is limited to 6 pages (or no more than 7 pages with USD \$100 over-length charge for the extra page). We strongly recommend incorporating reviewers' feedback when revising your paper. Please note that the paper title as well as the list and order of authors cannot be changed after the paper is accepted. The final camera-ready manuscript is due by 1 September, 2026. Detailed submission instructions will be given later and will be posted on the IEEE GLOBECOM 2026 website.

***Presentation Format and Session**

IEEE GLOBECOM 2026 includes both oral lecture-style presentation and interactive-style presentation sessions. This distinction has no relationship with the quality of the accepted papers whatsoever. You will receive a separate email informing you of the format (lecture or interactive) of your presentation, and the session to which your paper is assigned. Please note that by submitting a manuscript, all authors have implicitly agreed to present their accepted papers, regardless of format, in sessions organized by the Conference Program Committee. Authors may not request to change the presentation format of their accepted papers. To assist you with your presentation we suggest you review the presentation guide provided at the conference website.

***No-Show Policy**

The organizers of IEEE GLOBECOM 2026 as well as attendees expect an accepted paper to be presented in person at the conference by an author of that paper. To be published in the IEEE GLOBECOM 2026 Conference Proceedings and to be eligible for publication in IEEE Xplore®, the paper must be presented by an author of that paper at the conference. A paper that fails to be presented by an author or an approved substitute will be removed from the final conference proceedings before uploading to IEEE Xplore®. No refund will be made to authors of these papers.

***Logistics**

We advise you to make your airline and hotel reservations as early as possible. We will be updating our website in the next two weeks with the detailed information.

Congratulations again for having your paper accepted at IEEE GLOBECOM 2026, a flagship conference of the IEEE Communications Society. We look forward to welcoming you to Macau, China, in December 2026.

Sincerely,
Igor Bisio
Track Chair
IEEE GLOBECOM 2026 - SAC: e-Health

Lin Dai
IEEE GLOBECOM 2026 Technical Program Chair
Matthew Valenti and Huiling Zhu
IEEE GLOBECOM 2026 Technical Program Co-Chairs

----- Review Reports -----

===== GC 2026-review 1 =====

> *** Relevance and timeliness: Rate the importance and timeliness of the topic addressed in the paper within its area of research.
Excellent (5)

> *** Technical content and scientific rigour: Rate the technical content of the paper (e.g.: completeness of the analysis or simulation study, thoroughness of the treatise, accuracy of the models, etc.), its soundness and scientific rigour.
Solid work of notable importance. (4)

> *** Novelty and originality: Rate the novelty and originality of the ideas or results presented in the paper.
Significant original work and novel results. (4)

> *** Quality of presentation: Rate the paper organization, the clearness of text and figures, the completeness and accuracy of references.
Excellent. (5)

> *** Strong aspects: Comments to the author: what are the strong aspects of the paper

The paper addresses an important and timely problem at the intersection of federated learning, multimodal AI, and healthcare privacy. The proposed OmniMed-FL framework is well-motivated and demonstrates strong experimental breadth through the evaluation of multiple LLMs, ViTs, and fusion strategies under non-IID federated settings. The manuscript is generally well written, the figures are informative, and the system-oriented perspective aligns well with the scope of the conference. The anti-collapse regularization strategy and benchmarking effort add practical value to the work.

> *** Weak aspects: Comments to the author: what are the weak aspects of the paper?

The main limitation of the paper is the heavy reliance on synthetic clinical text and partially synthetic image data, which reduces the clinical realism and limits the strength of the conclusions. While the multimodal performance improvements are impressive, the contribution is primarily an integration of existing methods rather than a fundamentally new algorithmic framework. In addition, some claims regarding deployment readiness and state-of-the-art performance are somewhat stronger than what is fully supported by the experimental setup.

> *** Recommended changes: Please indicate any changes that should be made to the paper if accepted.

The paper would benefit from a clearer discussion of the limitations associated with synthetic multimodal data and a more cautious interpretation of the clinical applicability of the results. The authors should also strengthen the federated learning analysis by briefly discussing communication overhead, scalability, and robustness considerations. Reducing some of the stronger claims and improving the consistency and accuracy of a few references would further improve the paper's credibility and presentation quality.

> *** Submission Policy: Does the paper list the same author(s), title and abstract (minor wording differences in the abstract are okay) in its PDF file and EDAS registration? (yes/no)

Yes

===== GC 2026-review 2 =====

> *** Relevance and timeliness: Rate the importance and timeliness of the topic addressed in the paper within its area of research.
Excellent (5)

> *** Technical content and scientific rigour: Rate the technical content of the paper (e.g.: completeness of the analysis or simulation study, thoroughness of the treatise, accuracy of the models, etc.), its soundness and scientific rigour.
Solid work of notable importance. (4)

> *** Novelty and originality: Rate the novelty and originality of the ideas or results presented in the paper. Significant original work and novel results. (4)

> *** Quality of presentation: Rate the paper organization, the clearness of text and figures, the completeness and accuracy of references. Well written. (4)

> *** Strong aspects: Comments to the author: what are the strong aspects of the paper

This paper proposes a multimodal federated learning framework (named OmniMed-FL) for multi-class clinical condition classification.

> *** Weak aspects: Comments to the author: what are the weak aspects of the paper?

At the end of Abstract, adding few sentences highlighting the main findings from the experimental results, preferably in a quantitative fashion, in terms of the results given in Table IV.

> *** Recommended changes: Please indicate any changes that should be made to the paper if accepted.

This paper proposes a multimodal federated learning framework (named OmniMed-FL) for multi-class clinical condition classification. This framework covers 18 model variants for five-class clinical condition classification. For evaluation, five benchmarks and eight VLM fusion strategies are considered, and it is demonstrated that integrating medical images with electronic health records is crucial for accurate diagnosis across decentralized networks. It is also proved that the proposed multimodal approach can significantly outperform the isolated, single-modality systems. For evaluation purpose, a controlled clinical dataset was designed and validated.

Comments:

(1) At the end of Abstract, adding few sentences highlighting the main findings from the experimental results, preferably in a quantitative fashion, in terms of the results given in Table IV.

(2) A justification of the choice of simulation parameters given in Table II would be desirable.

> *** Submission Policy: Does the paper list the same author(s), title and abstract (minor wording differences in the abstract are okay) in its PDF file and EDAS registration? (yes/no)

Yes

===== GC 2026-review 3 =====

> *** Relevance and timeliness: Rate the importance and timeliness of the topic addressed in the paper within its area of research.

Good (4)

> *** Technical content and scientific rigour: Rate the technical content of the paper (e.g.: completeness of the analysis or simulation study, thoroughness of the treatise, accuracy of the models, etc.), its soundness and scientific rigour.

Solid work of notable importance. (4)

> *** Novelty and originality: Rate the novelty and originality of the ideas or results presented in the paper. Significant original work and novel results. (4)

> *** Quality of presentation: Rate the paper organization, the clearness of text and figures, the completeness and accuracy of references. Well written. (4)

> *** Strong aspects: Comments to the author: what are the strong aspects of the paper

The paper addresses an important and practical healthcare problem by integrating multimodal data (medical images and clinical text) within a privacy-preserving federated learning framework. The proposed OmniMed-FL framework is good. The study considers realistic federated settings with non-IID data partitioning across multiple hospital clients, which enhances the practical relevance of the work. The anti-collapse training mechanism is a notable contribution and appears effective in maintaining class diversity during training. Experimental results demonstrate that multimodal fusion significantly outperforms unimodal approaches, highlighting the benefits of combining visual and textual clinical information.

> *** Weak aspects: Comments to the author: what are the weak aspects of the paper?

The evaluation appears limited to a single dataset and a single non-IID setting (Dirichlet $\alpha = 1.0$), which restricts conclusions regarding generalizability and robustness. The manuscript lacks comprehensive ablation studies

quantifying the individual contributions of the anti-collapse components, warm-start initialization, and RAG module. Comparisons with state-of-the-art multimodal federated learning methods are insufficient, making it difficult to assess the true novelty and competitiveness of the proposed framework.

> *** Recommended changes: Please indicate any changes that should be made to the paper if accepted.

The authors are advised to conduct detailed ablation studies to quantify the contributions of each anti-collapse component, fusion strategy, and warm-start initialization mechanism. Provide runtime, communication cost, memory consumption, and scalability analyses under varying numbers of clients and data distributions. Compare the proposed method against recent state-of-the-art multimodal federated learning approaches using consistent experimental settings.

> *** Submission Policy: Does the paper list the same author(s), title and abstract (minor wording differences in the abstract are okay) in its PDF file and EDAS registration? (yes/no)

Yes

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Sat, Aug 1, 2026 at 8:50 PM

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